

Submission to Together Queensland

Request that the union seek an independent psychosocial risk assessment of Switchboard Services, Logan Hospital, Metro South Health

Submitted by Cory Lea Shepherd, AO3 Switchboard Services, workplace delegate · 6 August 2026

1 What I am asking the union to do

I ask that Together Queensland request Metro South Health to commission an independent psychosocial risk assessment of Switchboard Services at Logan Hospital, under the *Managing the risk of psychosocial hazards at work* Code of Practice 2022 and consistently with ISO 45003, with the operators consulted.

I ask that the request be made by the union, signed by the appropriate officer, and raised with the Chief Executive and through the Health Consultative Forum or its equivalent. I am the workplace delegate and I am the person raising it, but I do not think it should be my request alone: the matters are collective, they affect operators who are still at work, and a request that belongs to the union will outlast whatever happens in my own circumstances.

I also ask the union to consider whether a work group should be determined and a health and safety representative election facilitated for the area. I am not seeking that role. I am asking that the room have one.

2 Why the work area needs it

Switchboard is the hospital's communications hub. Operators activate emergency codes and page clinicians, handle a high daily call volume including callers in distress, work across day, evening, night and weekend shifts, and after hours work without supervision present. Operators are also the first point of contact when information held elsewhere in the organisation (rosters, on-call arrangements, contact directories) is out of date, and they absorb the complaints that follow.

Matters that have been raised over several years, and which I understand continue, include:

- 2.1 rostering practice and the adequacy of recovery between and within shifts;
- 2.2 fatigue on high-volume days, and the extent to which staff manage it using their own personal leave;
- 2.3 how complaints and concerns raised by operators are acknowledged, answered and closed;
- 2.4 the absence of supervision and decision support after hours and on weekends;
- 2.5 operational changes taking effect without consultation with the operators affected;
- 2.6 the absence of any debrief or recovery step following emergency codes, deaths, or abusive calls.

To my knowledge no psychosocial risk assessment has ever been conducted for this work area, or for the wider line it sits within. I may be wrong about that, and an assessment is the way to find out.

3 My own position, stated plainly

My own matters are entwined with this, and I want the union to have that in front of it rather than discover it later. I sustained a psychological injury in June 2024. I have an appeal on foot in the Commission, and I am currently in a dispute with the Health Service about attendance, roster, leave and pay. I have been away from the workplace since late June 2026.

I raise that for two reasons, and neither is that I want something from this request. The first is that it is the evidence: what happened to me is one operator's experience of the same work, and I would not be able to describe the room accurately without it. The second is that it is how I know my colleagues need something like this. I have worked beside people managing the same demands with the same absence of supports, and I do not want another operator to arrive where I have arrived.

I am not asking the union to advance my dispute through this request, and I do not want the assessment used for that purpose. If it assists the union to make the request without reference to me at all, I would

prefer that. My concern is the room.

4 What I propose the union put forward

- 4.1 **An independent assessor**, external to the Health Service, psychologist-led, reporting to a level above the functions being assessed.
- 4.2 **The data reviewed**: call demand by hour, day and month; rosters audited against the fatigue rules; leave patterns read against demand; turnover and vacancies; the complaints record and response times; incident history; overtime; time on the queue between breaks.
- 4.3 **The People at Work survey**, the validated instrument developed with the Queensland regulator, available at no cost, offered to all operators with anonymity protected.
- 4.4 **Observation across the full operating cycle**, including night and weekend shifts and both high- and low-volume days, under a written protocol agreed in consultation.
- 4.5 **Interviews offered to every operator**, and exit interviews sought from operators who have recently left the area.
- 4.6 **A risk register shared with the operators**, not only with management, and controls selected in consultation with owners and dates.
- 4.7 **A review point at twelve months**, with a small set of indicators tracked: fatigue-rule compliance, complaint response times, and the leave-against-demand pattern.

5 Provider proposal already obtained

So that cost and availability are not unknowns, I asked two providers for indicative proposals. Psychgroup (Sarah Hellwege, Director and Principal Psychologist) has replied in writing. Their *Psychosocial Risk Infrastructure Audit* reviews the organisation's own data, surveys, reporting systems, claims, rosters, policies, and interviews stakeholders across work health and safety, human resources, senior executives, health and safety representatives and workers, reporting against the Code of Practice and the ISO standards. The indicative cost is **\$10,000 to \$30,000** depending on scope and the number of interviews. A psychosocial risk assessment (an independently facilitated survey with qualitative interviews) is included at no cost within their Partner Program. Ms Hellwege invited the details to be passed to the relevant contact within the hospital for definitive scoping.

I have no association with the provider and no interest in who is engaged. Selection of an assessor is a matter for the Health Service in consultation. I obtained the proposal only so that the request could not be answered with uncertainty about cost or availability.

6 Material available to the union

I have prepared, as a worker's contribution, a psychosocial hazard mapping for the work area, a set of proposed controls, and a proposed assessment methodology. They are offered to the union to use, adapt or discard entirely as it sees fit. Nothing in them is a claim against any individual.

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